**Document No.:** LRK-PI-AUR
**Title:** Aurivex (cefovatrin) for oral suspension: Prescribing Information
**Product:** Aurivex

**Document type:** Prescribing Information
**Version:** 3.0
**Effective date:** 2025-09-01

FICTIONAL DOCUMENT FOR SOFTWARE DEMONSTRATION ONLY. Larkmoor Therapeutics, Aurivex and cefovatrin do not exist. Nothing in this document is real medical information.

# AURIVEX (cefovatrin) for oral suspension

## 1 INDICATIONS AND USAGE

Aurivex is a cephalosporin antibacterial indicated in pediatric patients 3 months to 12 years of age for the treatment of the following infections caused by susceptible bacteria:

- Acute otitis media caused by Streptococcus pneumoniae, Haemophilus influenzae or Moraxella catarrhalis.
- Pharyngitis or tonsillitis caused by Streptococcus pyogenes.
- Uncomplicated skin and skin structure infections caused by Staphylococcus aureus (methicillin-susceptible isolates only) or Streptococcus pyogenes.

To reduce the development of drug-resistant bacteria, use Aurivex only to treat infections proven or strongly suspected to be caused by bacteria.

## 2 DOSAGE AND ADMINISTRATION

### 2.1 Recommended dosage

| Infection | Dosage | Duration |
|---|---|---|
| Acute otitis media | 7 mg/kg every 12 hours | 10 days |
| Pharyngitis or tonsillitis | 14 mg/kg once daily | 10 days |
| Uncomplicated skin and skin structure infections | 7 mg/kg every 12 hours | 7 days |

The maximum dose is 600 mg per day. Aurivex may be given with or without food.

### 2.2 Dose by body weight (7 mg/kg every 12 hours)

| Body weight | Dose every 12 hours | Volume of 125 mg/5 mL | Volume of 250 mg/5 mL |
|---|---|---|---|
| 5 kg | 35 mg | 1.4 mL | 0.7 mL |
| 10 kg | 70 mg | 2.8 mL | 1.4 mL |
| 15 kg | 105 mg | 4.2 mL | 2.1 mL |
| 20 kg | 140 mg | 5.6 mL | 2.8 mL |
| 25 kg | 175 mg | 7.0 mL | 3.5 mL |
| 30 kg | 210 mg | 8.4 mL | 4.2 mL |
| 35 kg | 245 mg | 9.8 mL | 4.9 mL |
| 43 kg or more | 300 mg | 12 mL | 6 mL |

For once-daily dosing (14 mg/kg), give double the volume once a day, up to 600 mg.

### 2.3 Dosage in renal impairment

For patients with creatinine clearance below 30 mL/min/1.73 m², give 7 mg/kg once daily. Aurivex is removed by hemodialysis; give a dose at the end of each dialysis session.

### 2.4 Reconstitution

Tap the bottle until all the powder flows freely. Add the volume of water below in two portions, shaking well after each portion.

| Strength | Bottle size | Water to add |
|---|---|---|
| 125 mg/5 mL | 60 mL | 38 mL |
| 125 mg/5 mL | 100 mL | 63 mL |
| 250 mg/5 mL | 60 mL | 39 mL |
| 250 mg/5 mL | 100 mL | 66 mL |

After reconstitution:

- Store in a refrigerator at 2°C to 8°C, or at room temperature up to 25°C.
- Shake well before each use.
- Discard any unused portion after 10 days.

## 3 DOSAGE FORMS AND STRENGTHS

Powder for oral suspension: 125 mg/5 mL and 250 mg/5 mL after reconstitution. Strawberry-flavored, off-white to pale pink suspension.

## 4 CONTRAINDICATIONS

Known allergy to cefovatrin or to other cephalosporins.

## 5 WARNINGS AND PRECAUTIONS

### 5.1 Hypersensitivity reactions

Serious and occasionally fatal hypersensitivity (anaphylactic) reactions have been reported with beta-lactam antibacterials. Before therapy, ask whether the patient has had previous hypersensitivity reactions to cephalosporins, penicillins or other drugs. Cross-hypersensitivity among beta-lactams occurs in up to 2% of patients with a history of penicillin allergy. Do not use Aurivex in patients with a history of an immediate, severe reaction to penicillin, such as anaphylaxis, angioedema or urticaria.

### 5.2 Clostridioides difficile-associated diarrhea

C. difficile-associated diarrhea (CDAD) has been reported with nearly all antibacterial agents, including Aurivex, and may range from mild diarrhea to fatal colitis. Consider CDAD in all patients who present with diarrhea after antibacterial use.

### 5.3 Sucrose content

Each 5 mL of reconstituted suspension contains 2.9 g of sucrose. Take this into account in patients with diabetes mellitus.

### 5.4 Laboratory test interactions

A false-positive reaction for ketones in the urine may occur with nitroprusside tests. A positive direct Coombs test may occur.

## 6 ADVERSE REACTIONS

Table 2. Adverse reactions in at least 1% of pediatric patients treated with Aurivex (N=1,284)

| Adverse reaction | Aurivex (%) |
|---|---|
| Diarrhea | 8.3 |
| Diaper rash (in children using diapers) | 4.1 |
| Vomiting | 3.2 |
| Rash | 1.9 |
| Abdominal pain | 1.4 |
| Headache | 1.1 |

Discontinuation because of adverse reactions occurred in 2.2% of patients, most often because of diarrhea or rash.

## 7 DRUG INTERACTIONS

### 7.1 Antacids

Antacids containing aluminum or magnesium reduce the absorption of cefovatrin by about 40%. Give Aurivex at least 2 hours before or 2 hours after these antacids.

### 7.2 Iron supplements

Iron supplements reduce the absorption of cefovatrin by about 80%. Give Aurivex at least 2 hours before or 2 hours after iron supplements. Iron-fortified infant formula does not affect absorption to a clinically meaningful extent.

### 7.3 Probenecid

Probenecid increases cefovatrin exposure by about 60%. Coadministration is not recommended.

## 8 USE IN SPECIFIC POPULATIONS

### 8.4 Pediatric use

Safety and effectiveness in pediatric patients younger than 3 months of age have not been established.

### 8.6 Renal impairment

See 2.3.

## 12 CLINICAL PHARMACOLOGY

### 12.3 Pharmacokinetics

| Parameter | Value |
|---|---|
| Time to peak concentration | 1 to 2 hours |
| Elimination half-life (children 6 months to 12 years) | About 1.6 hours |
| Plasma protein binding | About 65% |
| Excreted unchanged in urine | About 85% |

## 16 HOW SUPPLIED, STORAGE AND HANDLING

| Strength | Bottle size | NDC |
|---|---|---|
| 125 mg/5 mL | 60 mL | 99107-420-60 |
| 125 mg/5 mL | 100 mL | 99107-420-10 |
| 250 mg/5 mL | 60 mL | 99107-425-60 |
| 250 mg/5 mL | 100 mL | 99107-425-10 |

Store the dry powder at 20°C to 25°C (68°F to 77°F). For storage after reconstitution, see 2.4.

## 17 PATIENT COUNSELING INFORMATION

- Shake the bottle well before each dose and measure the dose with the oral syringe or dosing cup provided, not a kitchen spoon.
- Finish the full course even if the child feels better.
- Contact the healthcare provider if the child develops watery or bloody stools, even up to two months after the last dose.

## REVISION HISTORY

| Version | Effective date | Changes |
|---|---|---|
| 3.0 | 2025-09-01 | 2.2 weight-based dosing table added; 7.3 probenecid added. |
| 2.2 | 2024-02-12 | 5.3 sucrose content updated. |

Medical Information: 1-800-555-0199, medinfo@larkmoor.example. To report suspected adverse reactions, contact Larkmoor Therapeutics Pharmacovigilance at 1-800-555-0177.

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